First Contact Practitioners (FCP) in Primary Care PT5000

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Learning outcomes

- Describe FCP in Physiotherapy
- Outline the historical, strategic, and political factors surrounding the implementation of first contact practitioners in Physiotherapy.
- Understand the key skills and responsibilities of FCP, including tools such as Screened’em and the international frameworks for red flag screening.
- Increase student’s employability for FCP Physio roles

What is a FCP?

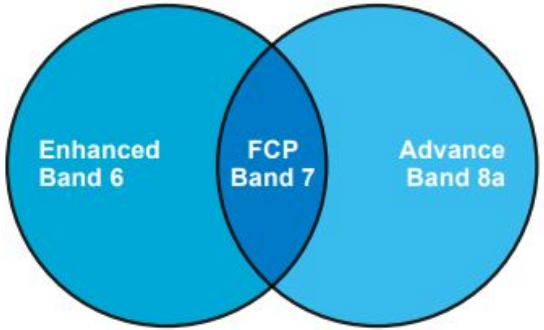
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What is a FCP?

- A regulated HCP who has undertaken additional training and experience within a specified scope of practice
- Example: MSK Physiotherapy, Dietetics, Occupational Therapy, Paramedics, Podiatrists
- To be the first point of contact and assessment for a defined group of patients
- Practitioners in FCP roles support people to get faster access to the right care without the need of referral from a GP
- FCP work apart of a multidisciplinary team

What is a FCP?

- Requires recognition through Health Education England
- The physiotherapist MUST have completed a taught or portfolio route
- The clinician must have a minimum of 5 years of post graduate experience (of which 3 in speciality area MSK)
- FCP work at masters level in their clinical pillar of practice



What do FCP Do?

- Specialist independent ax, diagnosis and management of undifferentiated undiagnosed MSK conditions
- Provide expert advice, education and preliminary management which may include individualised exercise programs
- Refer for and interpret any necessary investigations
- Refer for community/secondary care services (physio, hand therapy, orthopaedics, rheumatology etc)
- Promote health through lifestyle education
- Signpost to other services eg mental health, social prescribing
- MAY hold qualification to provide injection therapy
- MAY hold qualification to prescribe certain medications

History of FCP

- FCP roles began with the development of the FCP Physiotherapist in 2014 due to a shortage of GPs in Primary Care
- THE most important document to understand for FCP
- Stage 1 - to be completed PRIOR to employment
- Stage 2 - ongoing portfolio in practice
- Subsequent roadmaps for different professions

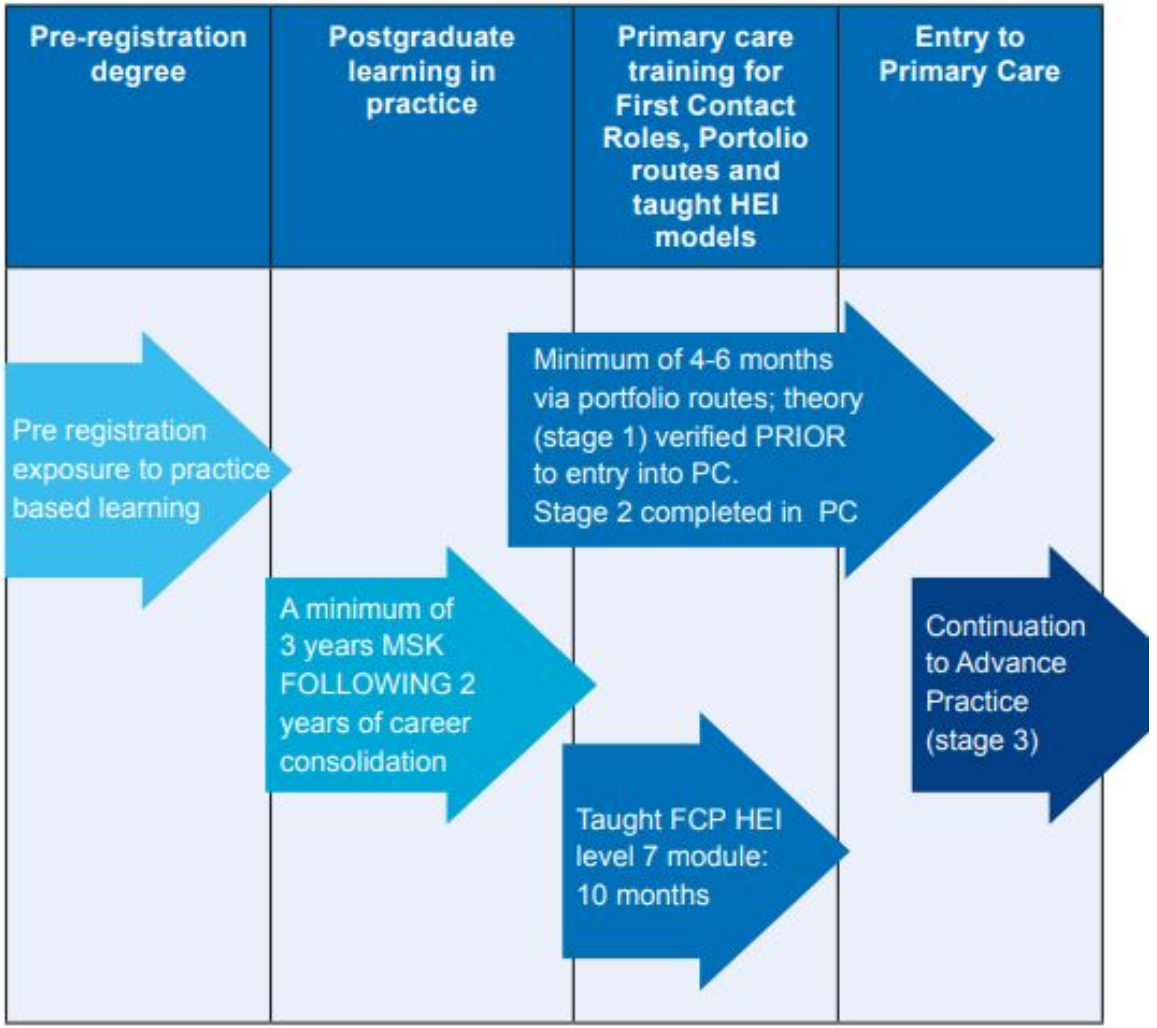
How can I be a FCP?

- Underpinned by guidance documents which set out nationally agreed education and training standards for AHP moving into FCP roles.
- FCP roadmaps were developed for each specific professions which underpin the education and training pathways
- **Framework: First contact practitioners and Advanced Practitioners in Primary Care: (MSK) A road map to practice**

FCP A ROADMAP TO PRACTICE



Career Progression



Key Skills

1. **Appropriate subjective questioning**
   - General health red flags
   - Spinal red flags
   - Rheumatoid red flags - INCLUDING SCREENDEM
   - Peripheral Joint Special Questions
2. **Appropriate objective assessment:**
   - Face to face
   - Neuro skills
   - Systematic Approach

Red flags

[POSITION STATEMENT]

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International Framework for Red Flags for Potential Serious Spinal Pathologies

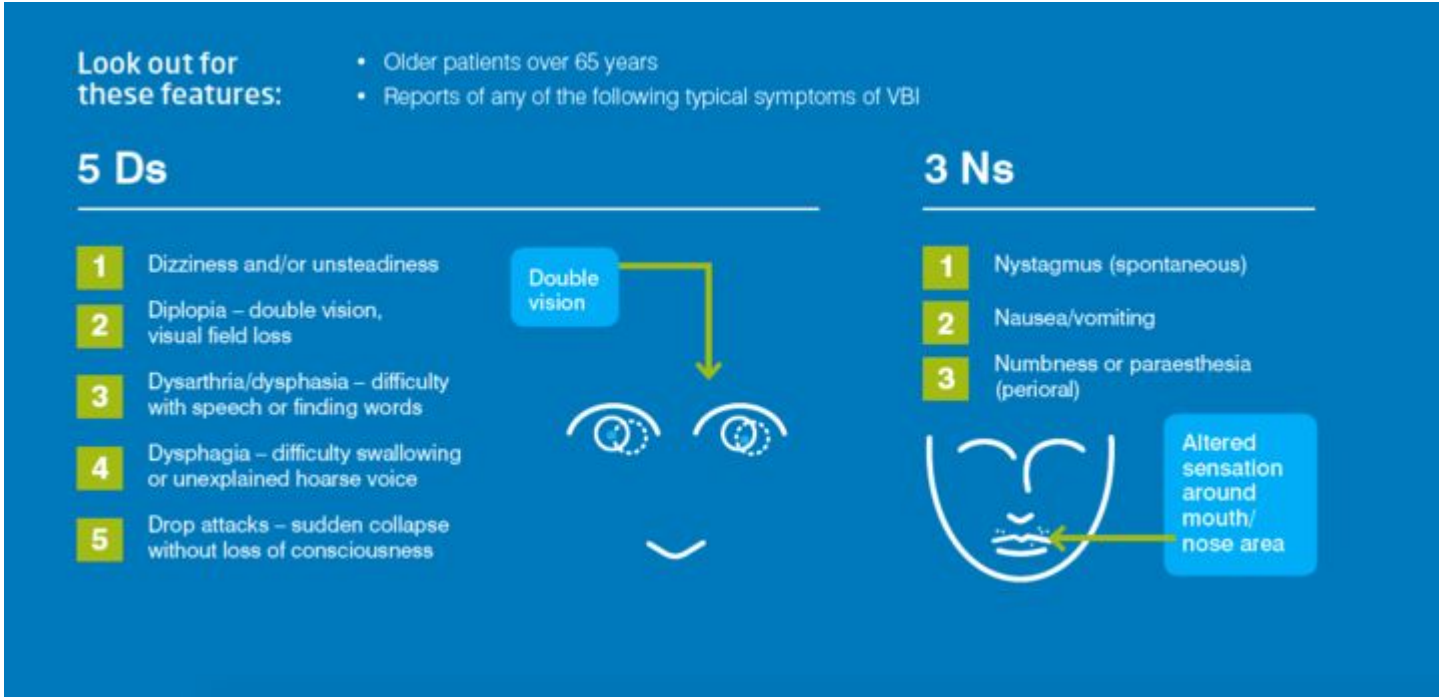
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Identifying serious pathology as the cause of a person's musculoskeletal presentation is complex. Red flags have historically been used to help clinicians identify serious spinal pathology, and the majority of guidelines recommend the use of red flags. However, guidelines are not consistent about which red flags should be considered when examining people seeking care for musculoskeletal disorders. This has led to confusion and inconsistency in the management of people when there is suspicion of serious pathology, and, in some cases, to unnecessary and worrying medical tests or false reassurance that there is no serious pathology. We aim to provide clinicians with a framework that includes academics and clinicians can masquerade as musculoskeletal spinal conditions. The framework has been informed by available evidence and augmented by a formal consensus process skeletal services can play an important role in early identification of serious pathology, ensuring that people achieve the best possible outcome. The prevalence of serious pathology will vary depending on where in the clinical pathway the clinician has contact with the person. Spinal surgeons likely see more cases of serious pathology than general practitioners do, and physical therapists probably see a number in between, depending on where they are on their clinical pathway. Therapists working at an advanced practice

Red flags:

1. Cauda Equina Syndrome
2. Fracture (osteoporituc compression fracture)
3. Fracture traumatic
4. Malignancy
5. Infection

Cervical Region Red flags



International Framework for Examination of the Cervical Region for Potential of Vascular Pathologies of the Neck Prior to Orthopaedic Manual Therapy (OMT) Intervention



Screen'ND em

- Memory aid to help remember the extra articular features of spondyloarthropathies
- E.g. ankylosing spondylitis, psoriatic arthritis, reactive arthritis

SCREEND'EM
BEFORE YOU TREAT'EM

A clinical tool to help identify spondyloarthropathy (SpA) in patients with tendinopathy.

SKIN

6-42% of patients with psoriasis develop psoriatic arthritis.



COLITIS OR CHROHN'S



Arthritis is one of the most common extra-intestinal manifestations of inflammatory bowel disease. The prevalence of SpA in patients with Chrohn's is estimated to be 26% at 6 year follow up.

RELATIVES



There is a strong relationship between SpA and HLA-B27 positive patients.

Family members of patients with SpA who are HLA-B27 positive have a 16-fold increase chance of developing ankylosing spondylitis if they are also HLA-B27 positive.

EYES



Acute anterior uveitis (AAU) can cause a painful, red eye with photophobia and blurred vision. 40% of patients presenting with idiopathic AAU have undiagnosed SpA. 50% of patients with AAU are HLA-B27 positive and >50% of these have SpA.

EARLY MORNING STIFFNESS

Inactivity related stiffness that lasts for more than 30 minutes is suggestive of inflammatory disease.

NAILS



Nail lesions occur in 87% of SpA patients and include

- small depressions in the nail (pitting
- thickening of the nails
- painless detachment from the nail bed (onchylosis)

DACTYLITIS

Sausage like swelling of the digits is a hallmark sign of psoriatic arthritis, occuring in 50% of cases.



ENTHESITIS

98% of SpA patients have at least one abnormal enthesis. The most common sites are the Achilles tendon, plantar fascia and patellar tendon.



MOVEMENT & MEDICATION EFFECT

SpA patients report improvement with activity but not with rest, and a favourable response to NSAIDs.

Created by Paul Kirwan

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THE KNEE RESOURCE

Key attributes:

- Advanced knowledge of neuromusculoskeletal systems + pathologies
- Advanced ax and diagnostic skills
- Excellent communication skills - patients + colleagues
- Awareness of limitations + MDT Working
- Professional curiosity - with pathologies but also social factors, safeguarding etc
- Knowledge of MSK prognosis - who can self-manage vs further treatment needed

Where do FCP work?

- In individual GP practices
- Group of GPs - Primary Care Network
- NHS trusts

Links to frameworks

- NHSE Putting Patients First
- New Deal for General Practice
- Future of Primary Care : Creating Teams for Tomorrow
- NHSE Vanguards
- Transforming MSK and Orthopaedic Elective Care *
- Elective High Impact interventions: First Contact practitioner for* MSK Services
- NHS Long Term Plan
- First Contact Roadmap to Pactice**
- International Framework for Red Flags **
- IFOMPT Cervical Framework**